ADR REPORTING & DOCUMENTATION

Pharmacovigilance Simulation & Safety Case Submission

Task ID: task2
Standard: CIOMS / CDSCO PV Form
Status: Ready for Review

1. CLINICAL CASE SUMMARY & NARRATIVE

A 48-year-old male outpatient (Initials: R.K., Weight: 68 kg) with a medical history of hypertension was initiated on oral Amoxicillin + Clavulanic Acid (625 mg BD) for lower respiratory tract infection on 20-Oct-2026. On Day 4 (23-Oct-2026), 45 minutes post-dose, the patient developed severe generalised urticaria, facial angioedema, and mild respiratory distress. The patient was transported to the Emergency Department, diagnosed with an acute anaphylactoid reaction, and treated with intramuscular epinephrine and IV hydrocortisone. The drug was permanently discontinued. All symptoms completely resolved within 24 hours under medical observation.

2. STRUCTURED PHARMACOVIGILANCE REPORT

SECTION A: PATIENT & SUSPECTED DRUG DETAILS		SECTION B: REACTION & CLINICAL MANAGEMENT	
Patient Initials	R. K.	**Adverse Reaction**	Generalised urticaria, facial angioedema, dyspnea
Age / Gender	48 Years / Male	**Onset Date / Latency**	23-Oct-2026 (~45 minutes post-dose)
Weight & History	68 kg | Hypertension	**Seriousness Criteria**	Medically Significant (Emergency intervention)
Suspected Medicine	Amoxicillin + Clavulanic Acid 625 mg	**Dechallenge Outcome**	Positive (Reaction abated upon withdrawal)
Dosage & Route	625 mg Oral Tablet (BD)	**Rechallenge Result**	Not re-introduced (Hypersensitivity hazard)
Dates of Therapy	20-Oct-2026 to 23-Oct-2026	**Final Reaction Outcome**	Recovered / Resolved on 24-Oct-2026
Concomitant Rx	Amlodipine 5 mg OD (Long-term)	**Emergency Treatment**	IM Epinephrine (0.5 mg), IV Hydrocortisone (100 mg)

3. MEDICAL CODING & REGULATORY CLASSIFICATION

Hierarchy / System	Classification Detail	Regulatory Notes
MedDRA SOC	Immune system disorders	Primary System Organ Class for hypersensitivity reactions
MedDRA PT / LLT	PT: Anaphylactic reaction LLT: Angioedema / Urticaria	Standardised MedDRA Query (SMQ): Anaphylaxis / Angioedema
Severity Grading	Grade 3 (Severe / Medically Significant)	Required urgent parenteral intervention to avoid impairment
Expedited Expediency	15-Day Expedited Alert Criteria	Fulfills standard CIOMS / CDSCO criteria for serious ADRs

4. CAUSALITY ASSESSMENT & PHARMACOVIGILANCE DECISION

WHO-UMC Causality Scale

Result: **Probable / Likely**

Consistent temporal sequence (45 min post-dose), positive dechallenge, and known immunogenic mechanism of beta-lactams without alternative confounding pathology.

Naranjo Algorithm Score

Total Score: **7 / 13 (Probable)**

Points credited for previous published evidence (+1), onset post-administration (+2), clinical recovery on withdrawal (+1), and absence of alternate etiology (+2).

5. REPORTER & ADMINISTRATIVE METADATA

Reporter Title	Pharmacovigilance Trainee / Intern	**Report Date**	08-Nov-2026
Affiliation	Department of Pharmacy Practice / PV Lab	**Report Type**	Initial Simulated Spontaneous Report